Cleveland Clinic

High Blood Pressure (Hypertension) During Pregnancy

Gestational hypertension is blood pressure greater than or equal to 140/90 that begins during the latter half of pregnancy (typically after 20 weeks). During pregnancy, high blood pressure can affect your body in different ways than it normally would. If high blood pressure goes unmanaged, both you and the fetus are at risk for complications.

Overview

What is hypertension during pregnancy?

Pregnancy-related hypertension is high blood pressure that begins after 20 weeks of pregnancy and ends shortly after your baby is born. It happens in about 6% to 8% of pregnancies. Your provider may also refer to high blood pressure during pregnancy as gestational hypertension or pregnancy-induced hypertension.

Gestational hypertension is different than other types of hypertension because it starts in the second half of pregnancy and goes away after childbirth.

High blood pressure may not cause any noticeable symptoms, so it's important that you attend all your prenatal appointments so your pregnancy care provider can take your blood pressure. High blood pressure during pregnancy can lead to complications. Your provider will monitor you and the fetus more closely and help you manage your blood pressure for the remainder of your pregnancy.

What is high blood pressure (hypertension)?

Blood pressure is the measurement of the pressure or force of blood pushing against blood vessel walls. When you have hypertension (high blood pressure), it means the pressure against the blood vessel walls is above the normal range.

How is high blood pressure during pregnancy different from high blood pressure at other times?

High blood pressure during pregnancy impacts your body differently than it would if you weren't pregnant. Your heart works harder during pregnancy because it has to pump much more blood. This puts extra stress on your body.

High blood pressure can also affect how the placenta develops and functions. This means the fetus may not get the nutrients it needs to grow at a normal rate. Both you and your child are at higher risk of complications before labor, during labor and after delivery if you have high blood pressure.

What are the different forms of high blood pressure you can have during pregnancy?

There are several different types of high blood pressure during pregnancy. They vary depending on when it begins and the symptoms it causes.

The most common types of high blood pressure during pregnancy are:

- **Chronic hypertension**: High blood pressure before pregnancy or early in pregnancy (before 20 weeks). This type of hypertension continues after your baby is born. People with chronic hypertension can also develop preeclampsia. This is known as chronic hypertension with superimposed preeclampsia.
- **Gestational hypertension**: High blood pressure in the latter part of pregnancy. Some people with gestational hypertension will go on to develop preeclampsia. Your provider will need to see you more frequently if you develop gestational hypertension.
- **Preeclampsia**: A condition only found in the second half of pregnancy (typically after 27 weeks of pregnancy). Your provider diagnoses this condition if you have elevated blood pressure readings and protein in your pee. It can affect your liver, kidneys, lungs or brain as well as the placenta. When it affects your brain, you're at risk for seizures (eclampsia).

Who is at higher risk of developing high blood pressure (hypertension) during pregnancy?

You may be more likely to develop high blood pressure during pregnancy if you:

- Are under age 20 or over age 40.
- Have had gestational hypertension or preeclampsia during past pregnancies.
- Have a family history of gestational hypertension.
- Have diabetes or gestational diabetes.
- Have an immune system disorder, such as lupus.
- Have kidney disease.
- Are expecting multiple babies (twins, triplets or more).
- Are Black.

Are gestational hypertension and preeclampsia the same?

No. Gestational hypertension is high blood pressure that occurs in the latter half of pregnancy but doesn't affect your kidneys or cause protein in your pee. This condition can advance to preeclampsia, so your provider will monitor your pregnancy more frequently.

Preeclampsia is a serious form of high blood pressure that also occurs in the latter half of pregnancy. Untreated preeclampsia can cause serious health problems like kidney, liver and brain damage. Preeclampsia can also cause growth problems for the fetus and increase the risk of stillbirth.

Symptoms and Causes

What causes high blood pressure during pregnancy?

The exact cause of hypertension during pregnancy is unknown. Certain conditions or factors may increase your risk (see above).

What are signs of high blood pressure during pregnancy?

High blood pressure is sometimes called "the silent killer" because most people don't notice their blood pressure is high. Other than a provider telling you that your blood pressure is high, some other signs are:

- Swelling (edema).
- Headaches.
- Sudden weight gain.
- Changes in vision.
- Nausea or vomiting.
- Peeing only a little bit at a time.
- Pain in your abdomen (belly).

What is the danger of high blood pressure (hypertension) during pregnancy?

Not everyone will develop complications from hypertension during pregnancy. However, high blood pressure puts you in danger of the following complications:

- Seizures.
- Stroke.
- Temporary kidney failure.
- Liver problems.
- Blood clotting problems.

You're also at higher risk of needing a C-section delivery if you have hypertension.

How does gestational hypertension affect my baby?

High blood pressure (hypertension) during pregnancy can prevent the placenta (the food and oxygen supply for a fetus) from receiving enough blood. This decrease in blood and nutrients can cause:

- Low birth weight or intrauterine growth restriction (IUGR). Low birth weight is when a baby is born weighing less than 5 pounds, 8 ounces.
- Premature birth (being born before 37 weeks of pregnancy).
- Stillbirth.
- Labor induction before your baby is full-term (39 weeks of pregnancy).

Most people who have high blood pressure will deliver healthy babies when the condition is caught early in pregnancy. The more severe the condition is, the more at risk you are for serious complications.

What is a normal blood pressure during pregnancy?

Your blood pressure is two numbers. The first number is your systolic blood pressure. It's a measurement of how you heart tightens or contracts. The second number is your diastolic blood pressure. This measures your heart when it's resting between beats.

Blood pressure is measured in millimeters of mercury, or mmHg. Your provider reads blood pressure as, "120 over 80," or systolic blood pressure over diastolic blood pressure.

It's important to see your provider during the first trimester so your typical blood pressure range can be checked. In the second trimester, blood pressure usually decreases, so it can be hard to detect chronic hypertension. If your blood pressure is over 140/90 mmHg during pregnancy, your provider may prescribe medication to control it. Systolic blood pressure over 160 mmHg or diastolic blood pressure over 110 mmHg can cause strokes or seizures, and may require treatment in a hospital setting during pregnancy.

Diagnosis and Tests

How is hypertension during pregnancy diagnosed?

Your pregnancy care provider takes your blood pressure at each prenatal appointment. Your blood pressure fluctuates throughout the day and many factors can make it rise.

If your provider sees any changes in your usual blood pressure readings or gets a higher-than-normal reading, they may suspect you have hypertension. Checking your blood pressure later in the appointment for a more normal reading is the first course of action. If your blood pressure doesn't stabilize, your provider may perform additional tests.

Other than additional blood pressure readings, your provider may also:

- Check your pee for protein.
- Check you for swelling.
- Perform blood tests to check liver and kidney function and blood clotting factors.

Management and Treatment

What is the treatment for gestational hypertension?

Healthcare providers treat high blood pressure during pregnancy in a variety of ways. Treatment depends on the severity of the condition, your overall health and how far along you are in pregnancy.

Your pregnancy care provider will want to monitor your blood pressure closely and check for symptoms of hypertension. This may mean more frequent prenatal visits to check your blood pressure and test your pee for signs of preeclampsia. Your healthcare provider may also recommend the following as part of your treatment:

- Monitoring your blood pressure at home.
- Medication to decrease your blood pressure.
- Hospitalization.

The goal of treatment is to reduce your blood pressure or prevent it from getting worse and causing complications.

They'll also want to monitor the fetus and perform frequent tests to check on its health. Some of these tests include:

- Ultrasound.
- Nonstress test.
- Biophysical profile.
- Doppler ultrasound (a type of ultrasound that measures blood flow).
- Counting your fetal movement (or kick counting).

If your provider feels the fetus will be born early, they may give you steroids that help mature its lungs. These steroids decrease the chance that your baby has breathing problems at birth.

What steps can I take to have a safe delivery with high blood pressure (hypertension)?

Steps that you can take to help the chances of having a safe and healthy delivery include:

- Taking any blood pressure medication as prescribed.

- Checking your blood pressure at home.
- Going to all of your prenatal visits.
- Having an early delivery if needed.
- Maintaining a healthy diet and consuming less salt.
- Following your healthcare provider's instructions regarding activity and exercise.

It's important to note that most people with hypertension don't have serious complications during pregnancy and delivery if their blood pressure is well-managed.

Will I be induced if I have gestational hypertension?

Possibly. If your pregnancy care provider feels it's safer to deliver early, they may recommend inducing labor. This could happen before your child is 37 weeks gestation (preterm).

Does bed rest help gestational hypertension?

Yes. Bed rest is one of the treatments your pregnancy care provider may recommend for treating hypertension during pregnancy.

Outlook / Prognosis

Will I still have high blood pressure (hypertension) after my baby is delivered?

High blood pressure (hypertension) during pregnancy typically goes away after your baby's delivered. However, it increases your risk of high blood pressure and heart disease in the future. If you had a severe form of preeclampsia or gestational hypertension, you may need medication for a few weeks after delivery. It's important to see your provider within 10 days postpartum (after your baby is born) if you had a severe form of high blood pressure at delivery.

People with chronic hypertension before pregnancy will usually still have the condition after delivery. Sometimes, blood pressure can remain high after delivery, requiring treatment with medication. Your healthcare provider will work with you after your pregnancy to manage your blood pressure.

When should I worry about gestational hypertension?

Call your pregnancy care provider if you experience any of the following symptoms:

- Sudden headaches that don't go away.
- Blurred vision or seeing double.
- Swelling.
- Peeing less than usual.

If you've been diagnosed with hypertension, talk to your pregnancy care provider about how you can best manage the condition. They can help make sure high blood pressure doesn't cause serious complications for you or the fetus.

Prevention

Can high blood pressure (hypertension) during pregnancy be prevented?

Since the cause of high blood pressure (hypertension) during pregnancy is unknown, it's hard to prevent it. The best thing you can do to prevent high blood pressure during pregnancy (and when you aren't pregnant) is:

- Maintain a healthy weight.
- Eat a healthy diet.
- Walk, ride a bike, practice yoga or do some other form of exercise several times per week.
- Avoid smoking and alcohol-containing beverages.
- Get plenty of rest.

If you're at risk for developing high blood pressure, your pregnancy care provider may recommend taking a baby aspirin each day. Understanding your risk of developing high blood pressure and knowing the warning signs may prevent serious complications of the disease.

Talk to your provider about any concerns you have about your blood pressure or risk for developing hypertension.

A note from Cleveland Clinic

Gestational hypertension, or high blood pressure during pregnancy, can cause serious complications for both you and the fetus. The good news is that this type of high blood pressure usually goes away after your baby is born. Treatment for high blood pressure involves more blood pressure readings, medication and lifestyle changes. When high blood pressure is well-managed, it doesn't cause serious problems. However, if left unmanaged, it can lead to premature birth, placental abruption or other serious complications. Speak to your pregnancy care provider if you have any concerns about your blood pressure readings or managing your blood pressure.

Experts You Can Trust

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Cleveland Clinic's health articles are based on evidence-backed information and review by medical professionals to ensure accuracy, reliability and up-to-date clinical standards.

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